

The Story

A brand manifesto, structured for graphics. Each (§) marks a new section/block. [GRAPHIC] tags suggest visual treatments.

§ 1 — Open cold. State the problem.

You've been sold a system that profits when you stay sick.

Not as a metaphor. Literally. The Western healthcare model — and the supplement industry tagging along beside it — is built around a simple, ugly incentive: a returning patient is worth more than a healed one. A symptom is worth more than a cure. A pill you take forever beats a problem you actually solve.

Once you see it, you can't un-see it.

[GRAPHIC: A clean split-screen. Left side: a revolving door labeled "the clinic." Right side: the same person walking through it again and again. Subtle, not preachy.]

§ 2 — The healthcare system isn't broken. It's working exactly as designed.

Modern medicine in North America was built on top of capitalism, which means every actor in the chain — insurance, pharma, hospitals, even most clinics — is optimizing for revenue, not for your long-term outcomes. It's not a conspiracy. It's just math.

That math leads to predictable results:

- **Treat symptoms, not causes.** A statin is recurring revenue. Teaching someone to fix their diet, sleep, and stress is a one-time conversation.
- **Prescribe, don't prevent.** Preventative medicine is barely reimbursed. Reactive medicine is where the money lives.
- **Speed over depth.** The average primary care visit in North America is around 15 minutes. That is not enough time to investigate root causes for anything that matters.
- **Glacial regulatory timelines.** It can take a decade and hundreds of millions of dollars to bring a single compound to market — which means only molecules with patent protection ever get the funding to even try.

This last point matters more than most people realize. Because it means: **the drugs you can legally buy are not the best drugs that exist. They're the best drugs that someone could profitably sell.**

[GRAPHIC: An iceberg. Above water: "what gets approved." Below water, much larger: "what works but isn't profitable enough to fund the trials."]

§ 3 — Case study: the ketamine problem.

Here is the cleanest example of how broken the system is.

Ketamine has two mirror-image forms — two enantiomers — called **R-ketamine** and **S-ketamine**. Same molecule, flipped. They behave very differently in the brain.

In 2019, Johnson & Johnson got FDA approval for an isolated **S-ketamine** nasal spray called Spravato for treatment-resistant depression. Big launch. Heavy marketing. Insurance coverage.

Here's what didn't make the press release:

- A 2020 meta-analysis pooling 24 trials and nearly 2,000 patients found that **racemic ketamine produced response and remission rates roughly twice as high as esketamine, with fewer dropouts.** ([Bahji et al., *Journal of Affective Disorders*](#))
- Preclinical research consistently shows **R-ketamine has longer-lasting antidepressant effects than S-ketamine, with fewer dissociative side effects and lower abuse potential.** ([Yang et al., *Translational Psychiatry*, 2015](#))
- The FDA's own internal meta-analysis put esketamine's effect size at **0.28 — comparable to or worse than older, cheaper drugs** like aripiprazole and quetiapine. The approval passed on a 14-2 advisory vote and was rejected outright by the UK's National Health Service.

So why is the worse molecule the one in pharmacies?

Because **racemic ketamine has been off-patent since the 1970s**. No company can recoup a 9-figure trial investment on a generic. **R-ketamine, the more promising enantiomer, has the same problem** — it's a known compound, hard to patent. The only way to make ketamine profitable was to isolate one half, repackage it as a "new" drug, and charge accordingly.

The result: the version with the strongest patent moat got the funding, the trials, and the green light. The version that probably works better is still stuck in early-phase research because no one can figure out how to make money from it.

This is not an isolated case. This is the operating system.

[GRAPHIC: Side-by-side molecule diagrams of R- and S-ketamine. One has a giant green checkmark labeled "Approved & profitable." The other has a question mark labeled "More effective, less patentable." The visual asymmetry IS the point.]

§ 4 — Now look at supplements. It gets worse.

If pharma has bad incentives but at least *some* gatekeeping, the supplement industry has neither.

Under the Dietary Supplement Health and Education Act of 1994 (the "DSHEA," for the United States) and the Natural Health Products framework in Canada, **supplements do not require FDA pre-market approval for safety or efficacy**. The manufacturer self-attests. The FDA only steps in *after* harm — after adverse event reports, after lawsuits, after recalls. Health Canada's process is slightly stricter on paper, but enforcement is similarly reactive.

Translation: the bottle on the shelf was not tested by the regulator before you bought it. The regulator is hoping you survive it.

What the data shows about what's actually in those bottles:

[GRAPHIC: A long, ugly stat block. Each line should hit. Consider stark typography, no decoration.]

- **47% of protein powders** tested in 2024–25 exceeded California's Proposition 65 safety thresholds for toxic heavy metals. ([Clean Label Project](#))
- **14% to 50%** of analyzed sport supplements have tested positive for anabolic agents or other prohibited substances they didn't disclose. ([Frontiers in Sports and Active Living, 2023](#))
- **More than 1 in 3** sports supplements purchased online didn't contain the key ingredients listed on their labels. ([Cohen et al., JAMA Network Open](#))
- **59% of botanical supplements** in one study contained plant species not listed on the label. **Four out of five products** from major retailers didn't contain the herbs they claimed.
- An older but jaw-dropping 2009 Government Accountability Office investigation found **trace contaminants in 93% of the 40 supplements they tested**.
- Between 2007 and 2016, the FDA identified **over 700 supplements adulterated with unapproved pharmaceutical drugs** — including hidden steroids, stimulants, and erectile dysfunction medications.
- In 2019 alone, dietary supplements were linked to **nearly 70,000 calls to U.S. poison control centers**.

You are, statistically, more likely to get something other than what's on the label than to get what's on the label.

This is the snake oil aisle. It just has nicer branding now.

§ 5 — The bioavailability scam (explained like you're five).

Here's the next layer of the con. Even when a supplement contains what it says, **the form of the ingredient often doesn't work.**

Imagine your body is a house. A nutrient is a piece of furniture. **Bioavailability** is whether the furniture actually fits through the door.

You can buy a beautiful sofa. But if it's bolted to a six-foot crate that won't fit through any doorway, it doesn't matter how nice the sofa is. It's not getting into your living room. You paid for it, it's sitting on your porch, and it's useless.

Most multivitamins do this on purpose. Why? **Because the cheap, useless form of a nutrient costs the manufacturer about 90% less than the form your body can actually use.** A few common offenders:

Nutrient	Cheap form (bad)	Bioavailable form (good)
Magnesium	Magnesium oxide	Magnesium glycinate or L-threonate
Vitamin B12	Cyanocobalamin	Methylcobalamin
Folate (B9)	Folic acid	L-methylfolate (5-MTHF)
Vitamin D	D2 (ergocalciferol)	D3 (cholecalciferol), with K2
Curcumin	Plain curcumin powder	Curcumin with piperine, or liposomal

Magnesium oxide, for example, has a bioavailability of around 4%. Magnesium glycinate is closer to 40%. **Same dose on the label. Ten times the actual effect.**

When a brand puts magnesium oxide in their multivitamin, they save pennies per bottle. You absorb almost none of it. The label still gets to say "100% Daily Value." Everyone wins. Except you.

[GRAPHIC: The sofa metaphor, drawn literally. Or a "what's actually getting in" visualization — a syringe of liquid where most of it spills out the side. Bonus: a price tag floating above the cheap form labeled "saves them \$0.04. costs you everything."]

§ 6 — The deficiency epidemic nobody's treating.

Now layer this on top:

- **Roughly half the global adult population is vitamin D deficient.** A 2024 systematic review and meta-analysis found 51.1% of healthy adults fall below the deficiency threshold — and adults aged 19–44 are among the most affected, because they supplement the least and spend the least time outside. (*Bratislava Medical Journal*, 2024)

- **Magnesium deficiency is widespread in Western populations** and directly impairs the body's ability to even use vitamin D. So the people most deficient in one are usually deficient in both, compounding.
- **The standard Western diet** — engineered by a food industry running the same playbook as pharma — is optimized for sales, not nutrition. It's calorie-dense, nutrient-poor, hyper-palatable, and addictive by design. High in processed fats, sodium, refined sugar. Low in everything that actually builds you.
- **These deficiencies aren't cosmetic.** Vitamin D deficiency is now linked in the literature to depression, cardiovascular disease, autoimmune disorders, and impaired immune function. Magnesium affects sleep, mood, hormone production, and stress response. Omega-3s affect cognition and inflammation. B12 affects energy, neurological function, and dopamine.

[GRAPHIC: A silhouette of a young person, with annotations pointing to the things being silently affected — mood, sleep, energy, hormones — and a small caption: "the average 25-year-old, deficient in 3+ key nutrients, told they're 'just stressed.'"]

Yes — diet, sleep, sun, and exercise are the foundation. Anyone telling you a pill replaces those is lying. **But the highest-leverage thing you can do tomorrow morning, with the lowest effort, is take the right supplement in the right form.** You are running a body on partially-empty fuel tanks. The fix is straightforward.

§ 7 — The quiet revolution: research is free, and AI just made it usable.

Here's the thing the old guard doesn't want you to internalize:

Almost every meta-analysis, every randomized controlled trial, every systematic review on every supplement and intervention you care about — is publicly available, right now, for free. PubMed. Cochrane. Google Scholar. The full text of the science.

For most of human history, the gatekeeper to this knowledge was a doctor — and most doctors haven't read the latest research on most topics, because they don't have the time. The average primary care physician would need to read **20+ hours of new papers a week** just to stay current in their field. They don't. Nobody does.

But now? **Artificial intelligence (AI) can synthesize the entire literature on a topic in minutes, for the price of a coffee.** A good model can pull twenty meta-analyses, weight them by quality, summarize the consensus, and flag the disagreements — in less time than it takes to book a doctor's appointment.

This doesn't replace clinicians. It does something arguably more important: **it removes the information asymmetry that the old system depended on.** You no longer have to trust. You can verify.

So the question becomes: if the research is free and the synthesis is cheap, **why is anyone still selling you a multivitamin without showing the receipts?**

[GRAPHIC: A simple diagram. Old world: patient → doctor → (gatekept knowledge).
New world: patient ↔ research ↔ AI. The arrows go both ways now.]

§ 8 — What we're building.

We're not going to pretend we invented supplements. We're going to do something simpler and rarer: **be honest about what's in the bottle, and prove it.**

[GRAPHIC: Four pillars or icons — clean, no fluff. These are the four things every section above has been building toward.]

1. Third-party tested. Every batch. Receipts public.

Every product we ship is tested by an independent, ISO-17025-accredited lab — not us, not our manufacturer, not someone we paid to look the other way. The certificate of analysis for your specific batch is on our site, scannable from the bottle. Heavy metals, microbial contamination, ingredient identity, ingredient potency. If it's not on the test, it's not in the bottle.

2. Only the bioavailable form. Always.

We will never use magnesium oxide. We will never use cyanocobalamin. We will never use folic acid where 5-MTHF belongs. We pay more per ingredient — sometimes 5-10× more — and we'd rather charge you fairly for something that works than cheaply for something that doesn't.

3. Every claim links to the paper.

Every product page links directly to the peer-reviewed meta-analyses and randomized controlled trials that support what we're selling. Not a marketing summary — the actual studies. If we can't cite it, we don't claim it. If the literature is mixed, we say so. We trust you to read.

4. Built for prevention. Priced for sustainability.

We don't want you on 14 supplements. We want you on the 3 or 4 your data and your life actually warrant — and we want you to eventually need fewer, not more. We are explicitly trying to put ourselves out of business with each customer. That's the goal. That's the only honest goal.

§ 9 — Close.

The healthcare system was built before the internet. Before open-access journals. Before AI. Before any consumer could verify what they were being told.

It made sense to trust gatekeepers when knowledge was scarce and synthesis was hard. Neither is true anymore.

You don't need a system that profits from your symptoms. You need clean inputs, real data, and a brand that treats you like an adult who can read.

That's what we're for.

[GRAPHIC: Closing visual — a single bottle on a clean background. Beside it, a QR code.
Caption: "Scan it. Read the test. Read the studies. Then decide."]

Notes for the team

- **Tone calibration:** We're confidently anti-status-quo without slipping into wellness-grift conspiracy energy. The whole brand depends on this distinction. We're the ones citing meta-analyses, not the ones screaming about Big Pharma.
- **Legal:** Anything that looks like a disease claim (treats depression, cures X) gets a compliance pass before it ships. Structure/function claims only on-label per DSHEA / Natural Health Products rules. The story above is brand narrative, not label copy — different rules apply.
- **Receipts > rhetoric:** Every section that lands hard does so because it has a real citation behind it. Keep that ratio when expanding.
- **The R-ketamine section is load-bearing.** It's the most concrete proof of the systemic argument. Don't cut it for length — cut elsewhere.